

CLIENT	DATE	VISIT TYPE
Deepak S.	2026-10-01	Initial Consultation (#1)
MODALITY	SUGGESTIBILITY	DURATION
Zoom (Virtual)	76% Physical	~60 min

ISSUES ADDRESSED

Presenting concern: "I am struggling to sleep because my mind is always busy, even when I am sleeping. I am actually sleeping longer but not well."

Assessment: Unrefreshing sleep despite adequate duration, consistent with insufficient deep (N3) sleep. Tracker shows 21% combined Deep+REM (does not separate N3 from REM). Recent international relocation presents significant novel-data processing load. Not anxiety-related. Prognosis: good.

SESSION METRICS (CLIENT-REPORTED BASELINE)

4/10	21%	79%	Bad
SLEEP QUALITY (SUBJECTIVE)	DEEP+REM COMBINED*	LIGHT SLEEP	TRACKER DEPTH SCORE

*Tracker data is for client's personal reference — therapy outcomes are measured through subjective experience and clinical observation.

TECHNIQUES USED

Eagle arm-raise, trophy staircase, cellular regeneration progressive, island cabin, Sorting Room, Depth Dial, Settling Pond

POST-HYPNOTIC SUGGESTIONS (PENDING — INSTALLATION IN SESSION 2)

Session 1 was cognitive assessment + first induction. PHS scheduled for installation in Session 2.

- 1. **Sorting Room:** Face Gallery, Language Wing, Map Room files daily data during delta sleep
- 2. **Depth Dial:** Surface (8 Hz) to DELTA LOCK (0.5 Hz), permanently installed
- 3. **Delta Floor:** Brain bypasses light sleep to delta within 30 min, 3-hour lock
- 4. **Settling Pond:** Day's impressions settle like gold dust in still water
- 5. **Island Cabin:** Permanent healing sanctuary, cedar, ocean air, cellular regeneration
- 6. **Pillow Trigger:** Head on pillow + 4-7-8 = Depth Dial auto-locks to DELTA
- 7. **Night-Waking Reset:** Hand on chest + one 4-7-8 = back to delta in 60 seconds
- 8. **10:30 PM Time Anchor:** Nervous system begins shutdown automatically
- 9. **Venting Dream Activation:** Vivid dreams = system clearing backlog
- 10. **Tracker as Convincer:** Rising deep sleep % reinforces programming

TREATMENT PLAN

Sessions: 6-8

Frequency: Every 1-2 weeks

Approach: Clinical hypnotherapy, Physical suggestible lane (direct/literal)

#	FOCUS	STATUS
1	Assessment + Education + First Induction	Completed
2	THE INSTALLATION — 10 PHS anchored in deep trance	Scheduled
3	Maintenance + Overactive Mind	Upcoming
4	Independence Pivot — self-hypnosis taught	Upcoming
5	Fine-Tuning	Upcoming
6	Graduation	Upcoming

PROGRESS INDICATORS

HOW WE MEASURE	WHERE YOU STARTED	WHAT IMPROVEMENT LOOKS LIKE
Morning restedness	4/10 (exhausted)	7-8/10 (refreshed)
Sleep onset ease	Difficult (~35 min)	Natural, effortless
Night-waking	Frequent, hard to return	Rare, returns easily
Daytime energy	Low, persistent fatigue	Consistent throughout day
Ceremony compliance	Starting	Effortless habit
Self-hypnosis confidence	Not yet learned	8+/10 (fully independent)

Progress is measured through subjective experience and clinical observation.

CLIENT RESPONSIBILITIES

NIGHTLY SLEEP CEREMONY (10:30 PM)	FREQ	DONE
Processing journaling (Wishful Thinking Stage)	Daily	--
Precognitive journaling	Daily	--
Venting with plan for next day/future	Daily	--
Close the journal	Daily	--
Breathing + self progressive relaxation	Daily	YES
Dream of deep peaceful sleep + self-hypnosis	Daily	--

CLIENT-REPORTED OUTCOMES

Post-session (Oct 2, 6:12 AM): Sleep 11 PM to 7:33 AM with only 1 bathroom wake. Heart rate dropped 72 to 63 bpm. Reports tendency to fatigue after 4-5 sessions.

NEXT STEPS

Schedule at: calendly.com/jeeth-vrishihypno/90min

If sleep deteriorates significantly or you experience persistent anxiety, panic, or suicidal thoughts — contact your primary care physician immediately.

SB 577 Disclosure: Jithendran Sellamuthu, C.MH. is not a licensed physician, psychologist, or psychiatrist. Services are provided for vocational/avocational self-improvement under CA B&P Code §2908. Not a substitute for medical/psychological treatment.

No Guarantee of Outcomes: Individual results vary based on suggestibility, compliance, and health. VRishi Hypnotherapy does not guarantee specific outcomes, cure rates, or timelines.

Therapeutic Exclusions: Clients on anti-depressant, anti-psychotic, or anti-anxiety medications, or with psychiatric history/suicidal ideation, must be referred to licensed mental health professionals.

Confidentiality: All information kept confidential per HIPAA Privacy Rule (45 CFR §§160, 164) and CA law, except where disclosure required by law.

Client Signature / Date

Therapist: Jithendran Sellamuthu, C.MH. / Date